PRACTICE EXAM 32 / EXAM SET 05

Domain 4: Patient Care Coordination and Education

12 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | Preventive Care and Care Gaps

> **SCENARIO** While preparing a preventive-care summary, an assistant finds two unresolved recommendations for the same patient's next colonoscopy. One signed clinician note specifies three years after the last procedure; another specifies five years. Neither note explains the difference, and no reconciled plan is documented.

How should the assistant establish an accurate tracking date?

A. Use the shorter interval as a cautious default without asking for further review.

B. Present both recommendations to the responsible provider for clarification before finalizing the reminder.

C. Use the longer interval because it reduces the number of procedures the patient must undergo.

D. Average the two recommendations and enter a four-year interval in the reminder system.

Q02 | Referrals and Care Transitions

> **SCENARIO** A signed referral order requests specialist evaluation within 48 hours. The electronic referral was transmitted with a routine-priority label, and the receiving office confirms it is in its routine triage queue. Clinic policy authorizes assistants to correct transmission fields to match the signed order and contact the receiving team about such errors.

Which action best restores the intended referral pathway?

A. Leave the routine label unchanged because the receiving office has acknowledged the referral.

B. Change the requested timeframe to match the receiving office's routine appointment schedule.

C. Tell the patient the signed request guarantees a specialist appointment within 48 hours.

D. Correct the transmitted priority, alert the receiving team, and confirm re-triage using the actual order.

Q03 | Patient Education and Self-Management Support

SCENARIO A provider's written nutrition plan limits a patient's total daily fluid to 1,500 mL. Its counting guide explicitly includes beverages, soup, and gelatin by the amounts consumed. By midafternoon, the patient has consumed 400 mL of water, 200 mL of tea, 150 mL of soup, and 100 mL of gelatin. The assistant asks the patient to apply the supplied guide.

Which calculation correctly identifies how much fluid remains in this patient's daily allowance?

A. 900 mL remains.
B. 750 mL remains.
C. 650 mL remains.
D. 850 mL remains.

Q04 | Community Resources and Access Barriers

SCENARIO A patient has arranged a taxi for an outpatient procedure but has no available support person. The procedure center's written policy requires an approved responsible adult to receive discharge instructions and accompany the patient home after the planned sedation. The center accepts a vetted escort program listed in the clinic directory, but no escort has been booked. The patient agrees to assistance.

Which coordination action best addresses the unmet requirement?

A. Confirm the escort program's availability and arrange an escort accepted by the procedure center.
B. Confirm the taxi reservation and treat the driver as the required discharge support person.
C. Ask the patient to sign a statement that replaces the center's responsible-adult requirement.
D. Tell the center that escort arrangements are complete because a program appears in the directory.

Q05 | Preventive Care and Care Gaps

SCENARIO Tomorrow's appointment is listed as a chronic-condition follow-up. The provider's previsit note also asks to discuss two outstanding preventive recommendations with the patient. Clinic workflow permits the assistant to prepare the verified care-gap summary and provider-approved education materials; screening orders and vaccination decisions remain with the provider.

Which previsit preparation best supports comprehensive care during this appointment?

A. Ignore the preventive recommendations because the appointment type lists a chronic condition.
B. Prepare the care-gap summary and approved materials so the provider can include the requested prevention discussion.
C. Place screening orders independently so the provider will not need to discuss the recommendations.
D. Mark the preventive recommendations addressed once the education materials have been printed.

Q06 | Patient Education and Self-Management Support

SCENARIO A patient correctly demonstrated use of the prescribed inhaler at an earlier visit. Two weeks later, during planned education follow-up, the assistant observes that the patient now misses steps on the same device-specific checklist. There is no new acute symptom, and the assistant is trained and authorized to reinforce the existing instructions.

Which follow-up strategy best supports retention of the taught skill?

A. Rely on the earlier successful demonstration and record that the teaching remains effective.
B. Provide another device booklet without practicing the missed steps or arranging a performance reassessment.
C. Reread the full checklist, ask whether the patient understands, and use agreement to close the education task.
D. Provide a focused refresher on the missed steps, recheck performance, and arrange another reinforcement check.

Q07 | Community Resources and Access Barriers

SCENARIO A patient recovering at home has enough groceries and can afford food but cannot prepare meals while both arms are temporarily restricted under the recovery plan. The patient asks for short-term help. A community program delivers prepared meals and can check its menu against the existing dietitian plan; a separate pantry distributes uncooked groceries.

Which resource connection most directly addresses the reported barrier?

A. Arrange additional uncooked groceries from the pantry without discussing meal preparation.
B. Give grocery coupons because the patient's stated problem is the price of food.
C. Verify suitable prepared-meal delivery and help arrange short-term service consistent with the existing plan.
D. Repeat the nutrition handout and ask the patient to practice cooking despite the recovery restrictions.

Q08 | Referrals and Care Transitions

SCENARIO A discharge plan identifies the patient's spouse as the trained helper for a prescribed home injection. During transition follow-up before the first home dose, the patient reports that the spouse is unavailable and an adult daughter will help instead. The daughter has not received the required teaching. Clinic policy routes changes in home-care readiness to the transition nurse for an authorized teaching arrangement.

Which information and action belong in the assistant's handoff?

A. Record that a family helper is available and consider the original caregiver teaching complete.
B. Report the helper change and training gap to the transition nurse to arrange teaching before the home task.
C. Recommend a different medication route so the daughter will not need injection teaching.
D. Send an unrelated injection video and document the daughter as trained without clinical follow-up.

Q09 | Community Resources and Access Barriers

SCENARIO A patient receiving ongoing clinical care for a chronic condition asks for a place to share experiences with other patients facing the same condition. The patient reports no new symptoms and is not asking to replace treatment or obtain caregiver training. The clinic directory includes an approved patient peer-support group, a caregiver workshop, and home delivery services.

Which resource best matches this patient's stated goal?

A. A caregiver skills workshop, because it is intended for people who provide another person's daily care.

B. A home grocery delivery service, because it can reduce the need to leave home.

C. Another diagnostic consultation, arranged without discussing the patient's request for shared experience.

D. The patient peer-support group, after confirming participation details and offering a connection alongside clinical care.

Q10 | Preventive Care and Care Gaps

SCENARIO A colorectal screening specimen produced a valid result, and the provider documented the screening as completed. The same provider reviewed the abnormal result and ordered diagnostic colonoscopy, which is still pending. The registry displays the screening as complete, but a separate diagnostic follow-up task remains open.

Which tracking approach accurately represents the patient's current status?

A. Erase the completed screening entry until the diagnostic colonoscopy is performed.

B. Retain the verified screening completion and continue tracking the ordered diagnostic follow-up separately.

C. Close the diagnostic task because a completed screening means the whole evaluation is complete.

D. Label the pending diagnostic task as refused because no completed procedure report is available yet.

Q11 | Referrals and Care Transitions

SCENARIO A signed physical therapy referral specifies the left knee. The receiving clinic's intake summary instead lists the right shoulder after a transcription error. The signed order is clear and unchanged. Clinic policy authorizes assistants to correct transmitted clerical details to match an existing order and verify the receiving record was updated.

Which action best corrects this referral discrepancy?

A. Send the order-matched correction under clinic policy and confirm that the receiving clinic updates the referral.

B. Ask the therapist to choose the body area from the inaccurate summary rather than the signed order.

C. Change the signed order to the right shoulder so it matches the receiving clinic's intake summary.

D. Leave the discrepancy for the patient to resolve during treatment without notifying the receiving clinic.

Q12 | Patient Education and Self-Management Support

> **SCENARIO** A patient correctly explains the prescribed medication schedule but reports missing doses because painful hand stiffness makes the bottle cap difficult to open. Cost and remembering the schedule are not problems. A young grandchild visits the home. The assistant is reinforcing the existing regimen and can arrange pharmacist assistance with medication-use barriers.

Which support best addresses the identified self-management problem?

A. Repeat the medication timing instructions and assume another explanation will resolve the opening difficulty.

B. Recommend leaving the bottle uncapped on the counter so each dose is easier to reach.

C. Arrange pharmacist review of suitable access aids or packaging options that also account for children in the home.

D. Recommend taking fewer scheduled doses so the patient will need to open the bottle less often.

ANSWER KEY & EXPLANATIONS

Practice Exam 32 | Domain 4: Patient Care Coordination and Education | 12 scored questions

01 B	02 D	03 C	04 A	05 B	06 D
07 C	08 B	09 D	10 B	11 A	12 C

DETAILED EXPLANATIONS

Q01 | Correct answer: B

Present both recommendations to the responsible provider for clarification before finalizing the reminder.

The record contains an unresolved clinical disagreement rather than a simple reminder-entry error. The assistant should preserve and present both recommendations for provider clarification instead of independently choosing, averaging, or replacing the screening interval.

Q02 | Correct answer: D

Correct the transmitted priority, alert the receiving team, and confirm re-triage using the actual order.

The urgency was already established by the provider; the coordination error is that the transmission did not carry it accurately. Correcting the fields and confirming re-triage restores that information without the assistant assigning new urgency or promising an appointment the receiving service has not confirmed.

Q03 | Correct answer: C

650 mL remains.

The counted intake is 400 + 200 + 150 + 100 = 850 mL; 1,500 - 850 = 650 mL remains. Counting only beverages gives 900 mL remaining, omitting gelatin gives 750 mL, and 850 mL is the amount already consumed rather than the remaining allowance.

Q04 | Correct answer: A

Confirm the escort program's availability and arrange an escort accepted by the procedure center.

A ride and an approved discharge escort serve different functions under the supplied center policy. Coordinating an accepted escort addresses the missing support, whereas a taxi booking or an unconfirmed directory listing does not establish that the requirement is met.

Q05 | Correct answer: B

Prepare the care-gap summary and approved materials so the provider can include the requested prevention discussion.

Previsit planning can make documented preventive needs visible and prepare useful materials during an already scheduled encounter. The appointment label does not remove the provider's prevention agenda, and preparing materials does not establish that counseling or a preventive service occurred.

Q06 | Correct answer: D

Provide a focused refresher on the missed steps, recheck performance, and arrange another reinforcement check.

A previously demonstrated skill may need reinforcement, and current observation identifies what to review. Focused reteaching with reassessment and follow-up addresses retention; earlier success or another booklet alone does not establish that the observed errors have been corrected.

Q07 | Correct answer: C

Verify suitable prepared-meal delivery and help arrange short-term service consistent with the existing plan.

The barrier is preparing available food, rather than food supply or affordability. A verified prepared-meal service addresses that function while preserving the established nutrition plan; more uncooked groceries or coupons would not solve the stated problem.

Q08 | Correct answer: B

Report the helper change and training gap to the transition nurse to arrange teaching before the home task.

The actual home helper differs from the person trained at discharge, creating a readiness gap that the transition nurse needs to address. Family relationship does not establish competence, and the assistant should not change the medication route or independently certify training from a generic video.

Q09 | Correct answer: D

The patient peer-support group, after confirming participation details and offering a connection alongside clinical care.

The requested support is connection with peers who have relevant lived experience. A matched peer-support group can meet that social need alongside ongoing treatment; caregiver training, delivery services, and additional testing serve different purposes.

Q10 | Correct answer: B

Retain the verified screening completion and continue tracking the ordered diagnostic follow-up separately.

Completion of the screening service and completion of its provider-ordered follow-up are different events. Keeping both statuses accurate preserves the verified service history and the unresolved diagnostic work without implying that the abnormal result has been fully addressed.

Q11 | Correct answer: A

Send the order-matched correction under clinic policy and confirm that the receiving clinic updates the referral.

The clinical order is unambiguous, and the supplied policy permits correcting a clerical transmission error to match it. Confirming the receiving update helps prevent the wrong service from being planned without changing the provider's order or making the patient responsible for correcting the referral.

Q12 | Correct answer: C

Arrange pharmacist review of suitable access aids or packaging options that also account for children in the home.

The barrier is physical access to the medication, so a pharmacist can help evaluate suitable dispensing aids or packaging while considering household child safety. Repeating a schedule the patient already understands does not address that barrier, and the assistant should not weaken storage protections or change prescribed dosing.

End of Practice Exam